Protocol for Treating Vulvodynia Using Low-Intensity Shockwaves



Device: MoreNovaPro - Li50F Linear Transducer

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1. Purpose and Scope

This protocol provides a standardized treatment plan for the use of low-intensity shockwave therapy to manage vulvodynia, a chronic vulvar pain condition without an identifiable infectious, dermatologic, or neurologic cause. It is intended for qualified healthcare providers using the MoreNova device with the Li50F transducer to target the vulvar vestibule and surrounding vulvar tissues. This protocol applies to females aged 18–70 years who are clinically diagnosed with provoked, unprovoked, or mixed vulvodynia and who have experienced persistent vulvar pain, burning, or discomfort for a duration of three months or longer.

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2. Introduction

Low-Intensity Extracorporeal Shockwave Therapy (LiESWT) is a non-invasive technology that delivers acoustic waves to stimulate tissue regeneration, enhance local blood circulation and support tissue remodeling, which may contribute to modulation of pain perception. In women with vulvodynia, LiESWT is applied to the vulvar vestibule and surrounding tissues with the aim of reducing chronic vulvar pain and burning sensations by improving microvascular function and supporting tissue remodeling. For further information on clinical rationale and emerging evidence, visit our Clinical Data Center.

Device Overview:

MoreNova is a portable Electromagnetic (EM) Low-Intensity Shockwave (LISW) device designed to stimulate angiogenesis, enhance microcirculation, and support tissue regeneration in targeted tissues, improving outcomes for various conditions in men and women.

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3. **Background Condition Overview**:

- Vulvodynia is a chronic vulvar pain condition characterized by burning, stinging, irritation, or rawness of the vulvar area, lasting three months or longer without an identifiable infectious, dermatologic, or neurologic cause.
- The condition may be provoked, unprovoked, or mixed, and is thought to involve altered peripheral nerve sensitivity, local inflammatory processes, and impaired microcirculation of the vulvar tissues.

Mechanism of Action

- Low-intensity shockwave therapy promotes neovascularization, enhances local blood flow, and supports tissue regeneration. These effects may contribute to reduced neurogenic inflammation, modulation of pain signaling, and improved tissue health in the vulvar vestibule, thereby alleviating chronic vulvar pain and discomfort.

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4. **Patient Selection Criteria Inclusion Criteria**:

- Women aged 18–70 years.
- Clinically diagnosed with vulvodynia (provoked, unprovoked, or mixed type) by a qualified gynecologist or vulvar pain specialist.
- Persistent vulvar pain, burning, stinging, or discomfort for ≥ 3 months.
- No identifiable infectious, dermatologic, or neurologic cause after appropriate evaluation.
- Symptoms refractory to or insufficiently managed by conservative treatments (e.g., topical therapy, oral medications, pelvic floor physical therapy).

Exclusion Criteria:

- Active vulvovaginal infection or inflammatory condition.
- Open wounds, ulcerations, or active dermatologic lesions in the vulvar or vestibular area.
- Recent vulvar, vaginal, or pelvic surgery (< 6 months).
- Known blood coagulation disorders or active anticoagulant therapy.
- Pregnancy or lactation.
- History of pelvic or external genital irradiation within the past year.
- Severe or uncontrolled chronic pain conditions or significant pelvic floor hypertonicity.
- Presence of implanted electronic or metal medical devices is sensitive to electromagnetic radiation.

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5. Pre-Treatment Screening

To determine the patient's suitability for LiESWT, conduct the following:

- Comprehensive medical, gynecological, and pain history, including symptom duration and triggers.
- Focused vulvar and vestibular examination to localize pain and exclude active pathology.
- Diagnostic evaluation as indicated to rule out infection or other identifiable causes (e.g., vaginal swabs, cultures, or biopsy when clinically warranted).
- Assessment of pain severity and functional impact using validated pain or vulvar symptom questionnaires, when available.

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6. **Treatment Protocol Preparation**:

- Conduct a thorough medical history and gynecological examination.
- Counsel the patient about the procedure, benefits, risks, and expected outcomes.

Procedure:

- **Number of Sessions**: 6-8 sessions based on severity.
- **Session Frequency**: Twice per week.
- **Session Duration**: 17 minutes. (2 passes around the vestibule)
- **Applications Per Session**: 2.

The rotation pattern:

1. Vulvar vestibule, right.
2. Vulvar vestibule, left.



- **Shocks Per Application**: 1000.
- **Total Shocks Per Session**: 2000.

Treatment Parameters:

- **Energy (HV) Settings**: 1.9 kV.
- **Frequency (FRQ)**: 2.0 Hz.

Post-Treatment Care:

- Advise patients to avoid sexual activity, tampon use, and other forms of local mechanical irritation for 24 hours following treatment.
- Recommend adequate hydration and gentle vulvar hygiene.
- Instruct patients to report any adverse events, including increased pain, skin irritation, or unusual discomfort.

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7. Treatment Areas and Techniques

The treatment focuses on the following anatomical areas:

- Vulvar vestibule, including the peri-introitus region.
- Right and left vestibular areas, corresponding to patient-reported pain localization.

Ensure correct Li50F transducer placement on the external vulvar tissues only. Apply gentle, uniform contact pressure using sufficient coupling gel to ensure acoustic transmission. Perform two circumferential passes around the vestibule, moving systematically to cover all designated areas in a controlled sequence. Avoid direct application over open skin, ulceration, or areas of acute inflammation.

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8. Patient Comfort and Safety

To ensure patient comfort and safety during the procedure:

- Position the patient in a supine or lithotomy position for optimal access.
- Use conductive gel for effective shockwave transmission and minimal discomfort.
- Continuously monitor patient feedback during the session and adjust settings if needed.
- Treatment should be discontinued if the patient experiences intolerable pain or adverse skin reactions.

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9. Treatment Outcomes and Follow-Up Expected Results:

Patients typically experience symptom improvement within 3–6 weeks of starting treatment. Regular assessments are recommended to evaluate progress.

Follow-Up Visits:

Schedule bi-weekly follow-ups to monitor patient outcomes and adjust the treatment plan as needed.

Long-Term Maintenance:

For sustained improvement, consider maintenance sessions every 3–6 months based on individual needs.

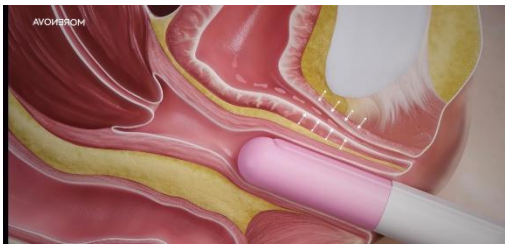
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10. Visuals and Diagrams

Li50F



- **Anatomical areas being treated.**



- **Rotation pattern for Transducer placement.**



11. **Quality Control Device Calibration**:

- Verify device settings before each session.
- The MoreNova Device recognizes preprogrammed parameters, ensuring correct settings are automatically applied.

Clinician Training:

- Ensure all operators are thoroughly trained in the use of the MoreNova Device and Li50F Transducer.
- Clinicians should be familiar with this protocol and review the "Instructions for Use" provided by the manufacturer.

Checklist for Each Session:

- Confirm that the patient meets the inclusion criteria and has no contraindications.
- Ensure the device and Transducer are clean, sanitized, and function correctly.
- Confirm that the Transducer placement aligns with the anatomical areas specified in the protocol.

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12. **References**

For clinical studies and additional resources, visit the Initia Clinical Data Center.